NOVOGENIA · AKADEMIE

FREQUENTLY ASKED QUESTIONS
EAT HEALTHY BY YOUR GENES

Answers to typical client questions about the genetic nutrition analysis: nutrient metabolism, gene variants, and practical recommendations.

INHALT

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This FAQ collection answers the most common client questions about the genetic nutrition analysis — how genes influence nutrient metabolism, what MTHFR, CYP1A2 and other genes mean, and how practical recommendations are derived.

BASICS — GENES AND NUTRITION

Why does the same food affect people differently?

Because we metabolize nutrients genetically differently. The MTHFR gene, for example, determines whether your body can activate folic acid into the bioactive form. The CYP1A2 gene determines how fast you break down caffeine. Such variants change how a nutrient acts in YOUR body.

How many genes does the nutrition analysis consider?

About 60 nutrition-relevant gene variants. Among them: MTHFR (folate activation), CYP1A2 (caffeine breakdown), ACE (salt/blood pressure), VDR (vitamin D receptor), HFE (iron uptake), LCT (lactase persistence), and antioxidant defense genes (SOD, CAT, GPX).

Can the analysis tell me whether I should drink coffee?

Indirectly yes. The CYP1A2 gene shows whether you break down caffeine fast (Warrior) or slow. Slow breakers carry higher cardiovascular risk from caffeine — coffee is better avoided or replaced with decaf for the polyphenol benefits.

Is the analysis a substitute for a doctor or nutritionist?

No. It is an additional decision-making aid based on stable genetic data. Acute symptoms, diseases, or special diets always belong in the hands of an MD or registered dietitian.

KEY GENE TOPICS

What does an MTHFR defect mean for me?

MTHFR activates folic acid into methylfolate, the form your body can actually use. With a defect, you can take a lot of folic acid in a regular supplement and still have functional deficiency. Switching to methylfolate solves the problem.

I have an HFE variant — is iron supplementation dangerous for me?

It can be. Some HFE variants cause iron overload over decades — your body absorbs too much from food and stores it in liver, heart, joints. Iron supplements should only be taken if blood tests show a real deficiency.

Salt sensitivity (ACE) — does that mean I have to give up salt?

Not entirely. It means your blood pressure responds more strongly to salt than average. Reducing salt is a powerful lever in your case — much more so than for non-sensitive people.

PRACTICAL QUESTIONS

Should I stop eating my favourite foods if they are rated unfavourable?

No. The report is a long-term guide. Occasional consumption is fine. The goal is balance over weeks and months, not perfection on a single day.

How fast will I notice an effect?

It depends on what you change. Caffeine sensitivity feels different within days. Folic-acid switching shows in 2-3 months on blood markers. Long-term recommendations like reducing salt or sat. fats take 6-12 months to translate into measurable cardiovascular benefits.

Is the recommendation list set in stone?

No. The recommendations are derived from current scientific consensus. As research progresses, the report is updated periodically with no extra cost.